

⚠ Addended Report: Review for Additional Results/Information

SURGICAL PATHOLOGY ORDER: S25-080994

Order:

Collected 13:58 Status: Edited Result - FINAL Dx: Abnormal finding on imaging

Test Result Released: No

Component

Final Diagnosis

- 1. Breast, Left; us left breast biopsy 7:00 periareoler; US BREAST CORE BIOPSY LEFT:
 - Invasive carcinoma type
 - Invasive mammary carcinoma
 - Tumor size
 - Invasive carcinoma spans at least 3.5 mm in this material
 - Differentiation
 - Moderately to poorly differentiated
 - Results of immunohistochemical studies
 - Will be reported in an addendum

I ATTEST THAT THE ABOVE DIAGNOSIS IS BASED UPON MY PERSONAL EXAMINATION OF THE SLIDES (AND/OR OTHER MATERIAL), AND THAT I HAVE REVIEWED AND APPROVED THE REPORT.

DILIP D GIRI, MD

Electronically signed by Dilip D Giri, MD on at 1312 EDT

Synoptic Checklist

Breast Biomarkers ((Added in Addendum) BREAST BIOMARKERS - All Specimens)

Results of Special Studies	Invasive carcinoma (block): 1C
Estrogen Receptor (Clone SP1, Ventana)	Positive - Tumor cells with nuclear staining: 99% % Strong intensity
Progesterone Receptor (Clone 16, Leica)	Negative (<1% nuclear staining)
HER2 (4B5; Ventana)	Negative (1+)
Comments	The tumor cells show loss of membranous expression of E-cadherin and p120. The findings are in favor of a diagnosis of invasive lobular carcinoma.

Addendum 01

This addendum reports the results of biomarkers performed on part 1. Please see the form for additional details.

Addendum electronically signed by Dilip D Giri, MD on at 0810 EDT

Gross Description

- 1. Breast, Left; us left breast biopsy 7:00 periareoler
- The specimen is received in formalin, labeled "Breast Left", and consists of five pieces of irregular yellow fibrofatty tissue fragments, ranging from 1.0 cm to 2.5 cm in length and 2.5 x 1.6 cm in aggregate. The specimen is entirely submitted in three cassettes.

Summary of sections:
BRST - breast

Carlos Lopez, HT

Disclaimer

Slides may be evaluated utilizing whole slide imaging techniques (WSI) and artificial intelligence (AI) techniques; these are laboratory developed methods whose performance characteristics have been evaluated by Memorial Sloan Kettering Cancer Center, Department of Pathology and Laboratory Medicine (PFI 2514), and have been deemed acceptable for clinical use in accordance with federal requirements as stipulated by the Clinical Laboratory Improvement Amendments of 1988. The New York State Department of Health, Clinical Laboratory Evaluation Program has determined that WSI and AI techniques are not subject to their review or approval as laboratory developed tests. These laboratory developed tests have not been approved or cleared by the Food and Drug Administration; the Food, Drug, and Cosmetic Act of 1976 provides exemption for laboratory developed testing performed at the discretion of licensed independent practitioners.

All controls are satisfactory. Some of the immunohistochemistry and Insitu Hybridization tests were developed and their performance characteristics were determined by the Department of Pathology and Laboratory Medicine (PFI 2514). They have not been cleared or approved by the US Food and Drug Administration. The FDA has determined that such clearance or approval is not necessary. These tests are used for clinical purposes. They should not be regarded as investigational or for research. This laboratory is certified under the Clinical Laboratory Improvement Amendments of 1988 (CLIA '88) as qualified to perform high complexity clinical laboratory testing.

All Controls are satisfactory. Ventana's PATHWAY anti-HER-2/neu is an FDA-approved rabbit monoclonal primary antibody (clone 4B5) directed against the internal domain of the c-erbB-2 oncoprotein (HER2) for immunohistochemical detection of HER2 protein overexpression in breast cancer tissue routinely processed for histologic evaluation. Results are reported in accordance with the ASCO/CAP guideline recommendations for HER2 testing in breast cancer (J Clin Oncol. 2018 Jul 10;36(20):2105-2122). ER and PR are also rabbit monoclonal antibodies which are FDA-cleared.

Resulting Agency MEMORIAL HOSP FOR CANCER AND ALLIED DISEASES DEPT OF PATHOLOGY

Specimen Collected: 13:58 Last Resulted: 08:10

- Order Details
- View Encounter
- Lab and Collection Details
- Routing
- Result History

Scans on Order

Lab Result Document - Document on 8:10 AM

Result Care Coordination

Patient

Released

Not seen

Component SmartLink Info

Component [ID]		
LAB AP CASE REPORT	CASEREPORT	CASE REPORT
LAB AP REPORT FINAL DIAGNOSIS NARRATIVE [34]	FINALDX	FINAL DIAGNOSIS
LAB AP SYNOPTIC CHECKLIST [71]	SYNOPTIC	SYNOPTIC
LAB AP ADDENDUM 01	ADD1	ADDENDUM
MSK LAB AP GROSS DESCRIPTION	GROSSDES	GROSS DESCRIPTION
LAB AP ASR DISCLAIMER [769]		LAB AP ASR DISCLAIMER
BEAKERPDFRESULT [3616]	BEAKERPDFRES	BEAKERPDFRESULT

Using Base Names in SmartLinks

Every Lab component (for example, WBC) has a corresponding base name SmartLink to pull the component into your note. Above is the base name

To pull a lab component with a base name

.RESUFAST[TSH

To pull two different lab components, type both their base names separated by a comma:

.RESUFAST[TSH,WBC

If you're interested in seeing more than just the last result, type a colon after the base name results to show:

.RESUFAST[TSH:3

.RESUFAST[TSH,WBC:3

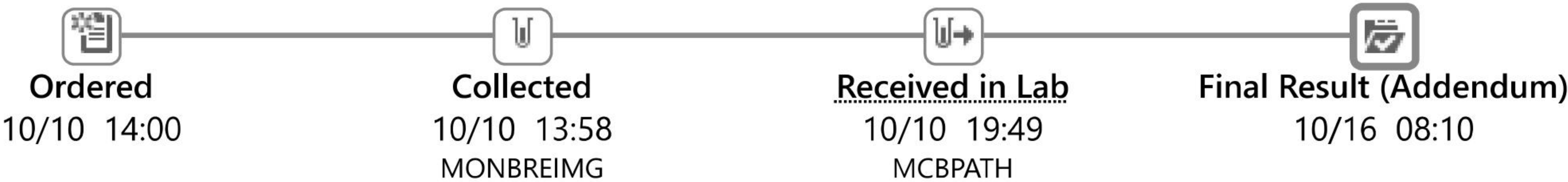
If you're interested in seeing all lab results from a given time period (for example, one month or six weeks), type a left-facing bracket after the .GETLABS SmartLink, followed by a number week:

.GETLABS[1M

.GETLABS[6W

The common name project team and isn't used in standard SmartLinks.

Order Tracking



Related Results

US BREAST DIAGNOSTIC BILAT Final result 1:03 PM

Left breast mass. Ultrasound-guided biopsy recommended.

BI-RADS 4 - Suspicious

RECOMMENDATION(S):
Ultrasound-guided breast biopsy- Left

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BI MAMMO TOMO DIAGNOSTIC BILAT Final result 12:34 PM

Left breast mass. Ultrasound-guided biopsy recommended.

BI-RADS 4 - Suspicious

RECOMMENDATION(S):
Ultrasound-guided breast biopsy- Left

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